

Foot MRI Dictation Cheat Sheet - Expanded Findings & Impressions

Copy-ready language. Replace bracketed text. Use as a workstation reference, not a substitute for surgical correlation or local report style.

Search Pattern

- Forefoot pain: plantar plate, neuroma/bursa, stress injury, MTP cartilage.
- Midfoot pain: Lisfranc complex, navicular stress fracture, coalition, midfoot OA.
- Lisfranc: inspect dorsal/interosseous/plantar components plus alignment and marrow edema.
- Sesamoid pain: fracture vs bipartite, AVN, sesamoiditis.
- Report exact ray/space because surgical planning depends on it.

Infection Search Pattern

- **1. Marrow/bone changes:**
 - Marrow replacement (darker than muscle/disc on T1; geographic pattern more predictive than reticular/periarticular).
 - Marrow edema (isolated finding raises concern in diabetics/high-risk).
 - Bone erosion (loss of dark T1/T2 cortical line).
- **2. Surrounding inflammatory changes:** fascia, musculature, subcutaneous fat edema/enhancement.
- **3. Susceptibility suggesting air:** in collections, soft tissues, along fascial planes, in bone.
- **4. Abnormal enhancement:**
 - Rim-enhancing collection = abscess.
 - Superficial lesion without enhancement = ulcer.
 - Enhancement along fascial planes = infectious/inflammatory fasciitis.
 - Non-enhancement of normal structures = ischemia.
 - Other patterns of enhancement may not be specific.
- **5. Differentiating osteomyelitis from neuropathic joint:**
 - Joint effusion with marrow replacement on both sides of joint.
 - Fluid collections/abscesses (periprosthetic, subperiosteal, intraosseous/Brodie's abscess).
 - Sinus tract connecting abnormal marrow signal to skin surface.
 - Disappearance of subchondral cysts on sequential imaging.
 - Progressive marrow/soft tissue changes greater than expected on sequential imaging.

Core Measurements

Structure	Normal / Threshold	Clinical Significance
Lisfranc interval	<2 mm between medial cuneiform and 2nd metatarsal base	>2 mm suggests ligament disruption; compare with contralateral
Metatarsal stress fracture risk	2nd and 3rd metatarsal shafts/necks	Most common sites; 2nd MT especially with hallux valgus transfer
Tibial sesamoid	10-12 mm long axis	Fracture if irregular margins + marrow edema
Fibular sesamoid	8-10 mm long axis	Less commonly injured; bipartite more common
Navicular body height	Loss of height = collapse	Central 1/3 is watershed zone — highest stress fracture risk

Foot MRI Tells

- **Lisfranc:** the dorsal component tears first; always check all three components (dorsal, interosseous, plantar). Interosseous is the strongest and most important.
- **Plantar plate:** look for crossover toe deformity — suggests chronic tear. Second MTP most commonly affected.
- **Morton neuroma:** between 2nd-3rd or 3rd-4th web spaces; T1 and T2 low signal mass (fibrotic). Not a true neuroma — perineural fibrosis.
- **Navicular stress fracture:** central 1/3 at highest risk (watershed vascular zone, tension side). Sagittal plane fracture on axial images.
- **Sesamoid AVN vs stress fracture:** dual sesamoids don't enhance uniformly. Bipartite sesamoid has smooth corticated margins; fracture has irregular edges with edema.
- **Freiberg disease:** 2nd metatarsal head most common. Flattening, sclerosis, and fragmentation suggest AVN.
- **Turf toe:** first MTP plantar plate/capsule injury. Grade the severity — grade 3 involves complete plantar plate tear with dorsal subluxation.

Modified Outerbridge Chondromalacia Grading - Copy-Ready

Use this for any articular surface. Replace [surface/compartment] with the specific location: patellar median ridge, medial femoral condyle weightbearing surface, anterosuperior acetabulum, radiocapitellar joint, tibiotalar dome, first MTP joint, etc.

Grade	MRI shorthand
0	Normal cartilage.
1	Signal heterogeneity/softening or swelling; surface intact.
2	Superficial fissuring/fraying/partial-thickness defect <50% cartilage depth.
3	Deep fissuring/partial-thickness defect >50% cartilage depth, not exposing bone.
4	Full-thickness cartilage loss with exposed subchondral bone, often edema/cysts.

Grade 0 - normal cartilage

F: Articular cartilage of the [surface/compartment] is preserved in thickness and signal without focal fissuring, delamination, or subchondral marrow abnormality.

I: No chondral defect of the [surface/compartment].

Grade 1 - chondral signal change / softening

F: Mild focal cartilage signal heterogeneity/softening of the [surface/compartment] with preserved cartilage thickness and intact articular surface. No subchondral marrow edema.

I: Grade 1 chondromalacia of the [surface/compartment].

Grade 2 - superficial partial-thickness chondral loss

F: Superficial chondral fissuring/fraying of the [surface/compartment] involving less than 50% of cartilage thickness. No full-thickness defect or subchondral marrow edema.

I: Grade 2 chondromalacia of the [surface/compartment].

Grade 3 - deep partial-thickness chondral loss

F: Deep partial-thickness chondral fissuring/defect of the [surface/compartment] involving greater than 50% of cartilage thickness without exposed subchondral bone. [Mild adjacent subchondral marrow edema.]

I: Grade 3 chondromalacia of the [surface/compartment].

Grade 4 - full-thickness chondral loss

F: Full-thickness cartilage loss/fissuring of the [surface/compartment] measuring [X] x [Y] mm with exposed subchondral bone and [subchondral marrow edema/cystic change]. [Small unstable chondral flap/loose body if present.]

I: Grade 4 chondromalacia/full-thickness chondral defect of the [surface/compartment].

Normal / Negative Templates

Normal foot

F: The metatarsals, phalanges, tarsal bones, and sesamoids demonstrate normal marrow signal without fracture, stress injury, or avascular necrosis. The Lisfranc ligament complex is intact with normal alignment of the tarsometatarsal joints. The plantar plates of the MTP joints are intact. No intermetatarsal mass or neuroma. The plantar fascia is normal in thickness and signal. The tendons and ligaments are intact. No joint effusion, synovitis, or soft tissue mass.

I: Normal MRI of the foot.

Useful Caveats

- **Bipartite sesamoid vs fracture:** bipartite has smooth, well-corticated margins with no marrow edema; fracture has irregular/jagged edges with adjacent marrow edema and soft tissue swelling. Bipartite is bilateral in ~25% of cases — contralateral comparison may help.
- **Accessory navicular types:** Type I is a sesamoid within the tibialis posterior tendon (os tibiale externum); Type II is connected to the navicular by a synchondrosis (most symptomatic); Type III is a cornuate navicular (fused prominence).
- **Coalition variants:** talocalcaneal (middle facet most common) and calcaneonavicular are the two most frequent. Look for irregular joint surfaces, marrow edema, and secondary signs (talar beak, ball-and-socket ankle).

Forefoot / Stress / Lisfranc

Plantar plate tear

F: [Partial/full-thickness] tear of the [2nd] MTP plantar plate at the phalangeal attachment with pericapsular edema and [dorsal subluxation/crossover deformity]. [Collateral ligament thickening/tear.]

I: [Partial/full-thickness] [2nd] MTP plantar plate tear. [Clinical correlation for crossover toe deformity.]

Morton neuroma

F: Ovoid low-to-intermediate T1 and T2 signal soft tissue mass in the [2nd-3rd/3rd-4th] intermetatarsal space measuring [X] mm in maximum transverse dimension, plantar to the intermetatarsal ligament. [Adjacent intermetatarsal bursitis.]

I: Morton neuroma in the [2nd-3rd/3rd-4th] intermetatarsal space.

Intermetatarsal bursitis

F: T2 hyperintense fluid distension of the [2nd-3rd/3rd-4th] intermetatarsal bursa dorsal to the intermetatarsal ligament, without discrete solid soft tissue mass. [Mild surrounding soft tissue edema.]

I: Intermetatarsal bursitis of the [2nd-3rd/3rd-4th] web space without Morton neuroma. [Consider reactive/inflammatory etiology.]

Metatarsal stress reaction

F: Marrow edema of the [2nd] metatarsal shaft/neck with periosteal edema but no discrete low-signal fracture line.

I: Stress reaction of the [2nd] metatarsal without completed fracture.

Metatarsal stress fracture

F: Linear low-signal fracture line through the [2nd] metatarsal [shaft/neck] with surrounding marrow and periosteal edema. [No displacement or angulation.]

I: Nondisplaced stress fracture of the [2nd] metatarsal.

Navicular stress fracture

F: Linear low-signal fracture line in the [central/dorsal] aspect of the navicular body on axial/sagittal images with surrounding marrow edema. [Partial/complete fracture extending from the dorsal cortex.] No displacement.

I: Navicular stress fracture. High-risk location — recommend non-weightbearing and orthopedic/sports medicine consultation.

Lisfranc ligament tear

F: Disruption/edema of the Lisfranc ligament complex between the medial cuneiform and second metatarsal base. The [dorsal/interosseous/plantar] component is [partially torn/completely torn]. Adjacent marrow edema in the medial cuneiform and second metatarsal base. [Diastasis of [X] mm between the medial cuneiform and second metatarsal base.] [Associated fractures of the [cuneiform/metatarsal base].]

I: Lisfranc ligament injury [with/without] malalignment. [Specify components involved.]

First MTP OA

F: First MTP osteoarthritis with cartilage loss, marginal osteophytes, subchondral cystic change/marrow edema, and small joint effusion/synovitis. [Dorsal osteophyte measuring [X] mm limiting dorsiflexion.]

I: First MTP osteoarthritis/hallux rigidus.

Sesamoid fracture/AVN

F: [Fracture]: irregular low-signal line through the [tibial/fibular] sesamoid with surrounding marrow edema and adjacent soft tissue edema. [AVN]: diffuse low T1 and heterogeneous T2 signal within the [tibial/fibular] sesamoid with [partial/absent] enhancement, fragmentation, and surrounding soft tissue edema.

I: [Tibial/fibular] sesamoid [fracture/avascular necrosis]. [Recommend correlation with contralateral foot to exclude bipartite variant if fracture is suspected.]

Accessory navicular (Type II) syndrome

F: Type II accessory navicular with a well-corticated ossicle connected to the navicular tuberosity by a synchondrosis. Marrow edema at the synchondrosis interface and within the accessory ossicle. Surrounding soft tissue edema and [tibialis posterior tendinosis/partial tear at the insertion]. [Small joint effusion.]

I: Symptomatic Type II accessory navicular with synchondrosis stress reaction and [tibialis posterior tendinopathy].

Tarsal coalition

F: [Fibrous/cartilaginous/osseous] coalition at the [talocalcaneal middle facet/calcaneonavicular] joint with irregular articular surfaces, [marrow edema at the coalition site], and [narrowing of the posterior subtalar joint]. Secondary signs include [talar beaking/ball-and-socket ankle joint/hindfoot valgus].

I: [Talocalcaneal/calcaneonavicular] coalition, [fibrous/cartilaginous/osseous] type. [Secondary degenerative changes.]

Freiberg disease

F: Flattening and irregularity of the [2nd/3rd] metatarsal head with subchondral sclerosis, fragmentation, and surrounding marrow edema. [Overlying cartilage loss and joint effusion.] [Loose bodies within the MTP joint.]

I: Freiberg disease (metatarsal head avascular necrosis) of the [2nd/3rd] metatarsal. [Smillie stage [I-V].]

Turf toe

F: [Grade 1: periligamentous edema surrounding the first MTP plantar plate/capsuloligamentous complex without discrete tear.] [Grade 2: partial-thickness tear of the plantar plate at the [proximal/distal] attachment with surrounding edema and small joint effusion.] [Grade 3: complete tear of the plantar plate with [proximal retraction/dorsal subluxation of the proximal phalanx] and [sesamoid diastasis/retraction].]

I: Turf toe injury, grade [1/2/3]. [First MTP plantar plate/capsuloligamentous tear.]

Midfoot osteoarthritis

F: Degenerative changes at the [tarsometatarsal/naviculocuneiform/intercuneiform] joints with cartilage loss, subchondral sclerosis, marginal osteophytes, and [dorsal osteophyte formation]. [Subchondral marrow edema suggesting active degenerative change.] [Small joint effusion.]

I: Midfoot osteoarthritis involving the [specify joints]. [Dorsal osteophyte impingement if symptomatic.]

Calcaneal stress fracture

F: Linear low-signal fracture line in the [posterior/anterior] calcaneal tuberosity perpendicular to the primary trabecular axis with surrounding marrow edema and periosteal edema along the calcaneal cortex. No displacement, cortical step-off, or extension to the subtalar joint surface. [Adjacent soft tissue edema along the Achilles insertion/plantar fascia origin.]

I: Nondisplaced calcaneal stress fracture. [Recommend clinical correlation, activity modification, and follow-up imaging if symptoms persist.]

Plantar fibromatosis (Ledderhose disease)

F: Fusiform/nodular thickening within the central cord of the plantar fascia measuring [X] x [Y] mm, demonstrating low-to-intermediate T1 and low-to-intermediate T2 signal. No aggressive features. [Multiple nodules along the plantar aponeurosis.]

I: Plantar fibroma/fibromatosis (Ledderhose disease). Benign. [Surgical excision if symptomatic and refractory to conservative measures.]

Source Notes

Built as a practical dictation aid from standard MSK MRI reporting patterns, review-level radiology references, and the existing shoulder cheat-sheet style. Use surgical correlation and local report conventions when needed.